



Reducing Adolescent Pregnancy in Oromia

An Evidence-Informed Policy Brief

Key Facts

- In Oromia, 17% of teenage girls aged 15 to 19 had begun childbearing; a rate higher than the national average of 13%. In some rural Oromia, it can reach up to 30.2%.
- Some of the important causes of teenage pregnancy include:
 - Child marriage-Oromia has one of the highest child marriage rates in the country (47%) next to Afar (67%) and, Somali (49%).
 - Being out of school and lack of education- Out of school teenagers in Oromia are over six times more likely to have a sexual experience compared to those attending school.
 - Living in rural areas- teenage girls in rural areas are three times more likely to have begun childbearing than those in urban areas
 - Being in the age group of 16-19- Adolescents between the ages of 16 and 19 years were 2.6 times more likely to have early risky sexual behavior when compared with those between the ages of 12 and 15 years
 - Being from a socio-economically poor family: 6% of women age 15-19 from the highest wealth quintile have begun childbearing, as compared with 24% of those from the lowest quintile.
 - Lack of knowledge of family planning
 - Weak enforcement of laws prohibiting child marriage complemented with poor birth registration practice hindering legal protection
 - Poor SRH services provision and monitoring, lack of contextualized strategies and guidelines as well as fragmented coordination across sectors
- **Actionable recommendations**
 - Policy-Level Recommendations
 - Multi-sectoral Collaboration; Contextualize National Roadmap for reducing teenage pregnancy; Strengthen Legal and Regulatory Frameworks; Strengthen Monitoring and Evaluation; Guidance on Digital and Social Media
 - Operational-Level Recommendations
 - Youth-Friendly SRH Services; Comprehensive SRH Education; Community Awareness and Attitudinal Change; Education and Empowerment Programs

Overview

Although Ethiopia's adolescent pregnancy rate has declined from 16.6% in 2005 to 12.5% in 2016 (1), it remains among the highest in Africa. In 2016, the adolescent birth rate for girls aged 15–19 was 145.97 per 1,000, substantially higher than the African average of 94.54 per 1,000 (2). High rates of adolescent pregnancy contribute to preventable maternal deaths, long-term health complications, and poor newborn outcomes, thereby undermining progress toward national and global

targets focused on maternal and newborn health (3). Within Ethiopia, Oromia bears one of the highest burdens of teenage pregnancy, underscoring the need for urgent, region-specific interventions. This evidence brief therefore aims to identify the underlying causes of teenage pregnancy in Oromia and to propose actionable recommendations based on the best available evidence from both a desk review and a primary study conducted in the region.

Causes

A combination of various factors play role as predictors of adolescent pregnancy in general. In Africa important predictors of teenage pregnancy were reported to be wealth index and marital status(4). High prevalence of teenage pregnancy is caused by the very high rates of child marriage practiced in regions of Ethiopia; 67% in Afar, 49% in Somali and 47% in Oromia, early marriage is an important factor for teenage pregnancy(2). According to a recent study in Oromia (5) age and sex of teenagers are also important factors for risky sexual behavior. Adolescents between the ages of 16 and 19 years were 2.6 times more likely to have early risky sexual behavior leading to teenage pregnancy when compared with those between the ages of 12 and 15 years. Sex was also a determinant factor where male participants were 63% less likely to engage in early risky sexual. Place of residence (rural/urban) is another contributing factor playing a significant role in teenage pregnancy in Ethiopia.

Teenage girls in rural areas are three times more likely to have begun childbearing than those in urban areas and 15% of rural teenagers have had a live birth or are pregnant, compared to just 5% of their urban counterparts(2).

Lack of knowledge of family planning has been associated with teenage pregnancy in Oromia(6).

School attendance or education is another important factor. Out of school teenagers in Oromia are over six times more likely to have a sexual experience compared to those attending school(5). Nationally data shows that teenage childbearing decreases with increasing levels of education. According to a national survey, the percentage of teenagers who have begun childbearing rises from 3% among those with more than a secondary education to 12% among those with only a primary education, and 28% among those with no education at all(2).

Other contributing factors for teenage pregnancy in Oromia include: weak enforcement of laws prohibiting child marriage and poor birth registration system; lack of social media guide, lack of knowledge of family planning and inadequate SRH services; lack of contextualized strategies/guidelines and fragmented cross-sectoral coordination; cultural acceptance of early marriage and resistance to SRH education; peer pressure and weak parental supervision; inadequate national policies contextualization to regional strategies/policies(5).

The Consequences

Adolescent pregnancy is a significant global public health concern with broad social, economic, and health implications. According to the World Health Organization (WHO), adolescents aged 15–19 years in low- and middle-income countries (LMICs) experience an estimated 21 million pregnancies annually as of 2019 (3). Of these pregnancies, 50% were unintended, and 55% of these unintended pregnancies among adolescent girls, aged 15–19 years, end in abortions which are usually unsafe in low-and middle-income countries (7). In addition to risks associated with unsafe abortions, adolescent mothers aged 10–19 are more likely to experience complications such as eclampsia, postpartum infections, and systemic illnesses compared to women aged 20–24 years. Additionally, their babies are at increased risk of being born prematurely, having low

birth weight, and suffering from serious neonatal health problems (3). As a result, complications related to pregnancy and childbirth are the primary cause of death among girls aged 15–19 worldwide with nearly all maternal deaths (about 99%) among women aged 15–49 occurring in low- and middle-income countries (8).

In addition to the serious health consequences mentioned above, pregnant adolescent girls face social and economic consequences too. Significant proportions of unmarried pregnant adolescents feel stigmatized by pregnancy and are at increased risk of social isolation and abuse (9). Adolescent pregnancy has significant economic consequences, primarily by reducing a mother's educational attainment and lifetime earnings, which can perpetuate a cycle of poverty and strain public resources (10).

Size of the Problem

In Oromia, 17% of teenage girls aged 15 to 19 had begun childbearing, according to the 2016 Ethiopia Demographic and Health Survey (EDHS). This rate is higher than the national average of 13%, but lower than in regions such as Afar (23%) and Somali (19%). According to additional studies, the frequency can reach up to 30.2% in rural Oromia (6). A systematic review in 2021 (11) has put the national prevalence of teenage pregnancy at 23.5%, hinting that the 13% national prevalence could be an underestimation.

Recommendations

Based on key findings, the following recommendations at the policy-level and operational-level are forwarded:

Policy-Level Recommendations

1. Multi-sectoral collaboration

- Short Term (<1 yr): The Oromia president's office should establish a Multi-Sectoral Task Force to coordinate cross-sector interventions. The establishment of this task force should consider key stakeholders from the Oromia Health Bureau, Oromia Education Bureau, Oromia Social Services Bureau, and Oromia Youth and Sports Bureau, among others.
- Mid-term (1–2 yrs): Following the task force establishment, the Oromia president's office should create a legal/policy framework (a guideline) for multi-sectoral operation.

2. **Adapt and contextualize the national Roadmap for reducing teenage pregnancy rates and improving reproductive health outcomes in Ethiopia (2025-2030)**
 - Short-term (<1 yr): The Oromia health bureau should adapt and endorse the National Teenage Pregnancy Reduction Roadmap for Oromia.
 - Mid-term (1–2 yrs): The Oromia Health Bureau to implement the adapted roadmap in collaboration with key bureaus and offices (including Education, Women and Children's Affairs Bureau).
3. **Strengthen Legal and Regulatory Frameworks**
 - Short-term (<1 yr): The Oromia Vital registration bureau should strengthen birth registration systems to provide legal evidence of age at marriage. Similarly, the *Oromia Attorney General Bureau should ensure the enforcement of existing laws against early marriage.*
 - Mid-term (1–2 yrs): The *Oromia Health Bureau, in collaboration with Oromia Investment Bureau and Oromia Civil Service Commission, should devise a means by which the government and business entities implement workplace-appropriate SRH strategies to reduce teenage pregnancy.*
4. **Strengthen Monitoring and Evaluation**
 - Short-term (<1 yr): The Oromia Health Bureau should enhance SRH policy implementation monitoring mechanisms at the regional and local levels.
5. **Guidance on Digital and Social Media**
 - Mid-term (1–2 yrs): The *Oromia Health Bureau, in collaboration with the Oromia President's office, should develop contextualized regional guidance on the use of social media for SRH information dissemination and youth engagement.*

Operational-Level Recommendations

1. **Youth-Friendly SRH Services**
 - Mid-term (1–2 yrs): The Oromia health bureau should strengthen and expand youth-friendly health services (YFS) at facilities by integrating YFS into facility design and construction, ensuring privacy, trained staff, respectful and culturally sensitive care, and active youth involvement in design and feedback.
 - Short-term (<1 yr): The bureau should also improve the availability and supply of SRH products to make them accessible to teenagers.
2. **Comprehensive SRH Education**
 - Mid-term (1–2 yrs): *The Oromia Health Bureau, in collaboration with the Oromia Education Bureau, should enhance and expand school-based SRH education through age-appropriate curricula by advocating for mandatory inclusion of SRH education in the curriculum, as well as training teachers and engaging parents/communities in SRH Education.*
3. **Integrate life skills, counseling, and contraceptives- access to reduce risky behaviors among adolescents. Community Awareness and Attitudinal Change**
 - Short-term (<1 yr): *The Oromia Health Bureau should strengthen community awareness campaigns to improve family–child communication on SRH, address stigma, and increase knowledge on prevention mechanisms. The bureau should promote community-driven mechanisms to encourage delayed sexual initiation and positive values (including abstinence).*
4. **Education and Empowerment Programs**
 - Long-term (2–3 yrs): *The Oromia Education Bureau, in collaboration with the Oromia Women and Children Affairs Bureau, should expand education and empowerment programs, including conditional cash or in-kind transfers combined with youth skills development initiatives, to keep adolescents, especially girls, in school and delay early marriage.*
5. **Social and Behavioral Change Interventions**
 - Mid-term (1–2 yrs): The Oromia health bureau should scale up youth-led approaches, peer networks, safe spaces, and digital platforms to challenge harmful norms, promote positive behaviors, and increase SRH service uptake.

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